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PRESMEDIC

Presmedic Ibuprofen 400mg Tablet

DESCRIPTION
Round, 13.0mm in diameter, pink coated tablet with shallow convex faces, bevel-edged and "HD"embossed on one face.

COMPOSITION
Ibuprofen 400 mg/tablet
This product contains Ponceau 4R as colouring agent.

PHARMACODYNAMICS
Ibuprofen is a propionic acid derivative NSAID that has demonstrated its efficacy by inhibition of prostaglandin synthesis. In humans, ibuprofen reduces inflammatory pain, swellings and fever. Furthermore, ibuprofen reversibly inhibits platelet aggregation.

PHARMACOKINETICS
Ibuprofen is rapidly absorbed following administration and is rapidly distributed throughout the whole body. The excretion is rapid and complete via the kidneys.

Maximum plasma concentrations are reached 45 minutes after ingestion if taken on an empty stomach. When taken with food, peak levels are observed after 1 to 2 hours. These times may vary with different dosage forms.

Elimination half-life is approximately 2 hours.

INDICATION
Presmedic Ibuprofen Tablet 400mg relieves pain such as headache, dental pain, period pain, rheumatic pain, muscular pain and backache.

Presmedic Ibuprofen Tablet 400mg relieves fever such as fever associated with cold and flu.

Presmedic Ibuprofen Tablet 400mg relieves inflammation of muscles and joints in rheumatic disease including juvenile arthritis and other muscular skeletal disorder.

CONTRAINDICATIONS
It should not be use in the following conditions:
- Hypersensitivity to ibuprofen or any of the excipients in the product.
- Patients who have previously shown hypersensitivity reactions (e.g. asthma, rhinitis, angioedema, or urticarial) in response to aspirin or other NSAID drugs.
- Active or history of recurrent peptic ulcer/haemorrhage (two or more distinct episodes of proven ulceration or bleeding).
- History of gastrointestinal bleeding or perforation, related to previous NSAIDs therapy.
- Severe heart failure (NYHA Class IV), renal failure or hepatic failure.
- Last trimester of pregnancy.

WARNING AND PRECAUTIONS
Warning

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID
- Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Precautions
- Caution in patients with anaemia or asthma, gastrointestinal toxicity, hypertension, congestive heart failure, diabetes mellitus, preexisting edema, extracellular volume depletion or sepsis, hemophilia or other bleeding problems, hepatic function impairment, renal function impairment, stomatitis, symptoms of bronchospasm, allergic rhinitis, urticaria, induced by aspirin or other NSAIDs, systemic lupus erythematosus (SLE).
- Blurred and/or diminished vision, scotomata, and/or changes in colour vision have been reported. If a patient develops such complaints while receiving Presmedic Ibuprofen, the drug should be discontinued and the patient should have an opthalmologic examination which includes central visual fields and colour visions testing.
- Fluid retention and edema have been reported in association with Presmedic Ibuprofen; therefore, the drug should be used with caution in patients with a history of cardiac decompensation or hypertension.
- Presmedic Ibuprofen, like other nonsteroidal anti-inflammatory agents, can inhibit platelet aggregation but the effect is quantitatively less and of shorter duration than that seen with aspirin. Ibuprofen has been shown to prolong bleeding time (but within the normal range), in normal subjects.
- Because this prolonged bleeding effect may be exaggerated in patients with underlying hemostatic defects, Presmedic Ibuprofen should be used with caution in persons with intrinsic coagulation defects and those on anticoagulant therapy.
- Patients on Presmedic Ibuprofen should report to their physicians signs or symptoms of gastrointestinal ulceration or bleeding, blurred visions or other eye symptoms, skin rash, weight gain, or edema.
- In order to avoid exacerbation of disease or adrenal insufficiency, patients who have been on prolonged corticosteroid therapy should have their therapy tapered slowly rather than discontinued abruptly when Presmedic Ibuprofen is added to the treatment program.
- The antipyretic and anti-inflammatory activity of ibuprofen may reduce fever and inflammation, thus diminishing their utility as diagnostic signs in detecting complications of presumed non inflammatory painful conditions.
- Patients hypersensitive to aspirin, to one of the non-steroidal anti-inflammatory agents, or to related analgesic agents may be hypersensitive to ibuprofen.
- Whilst no teratogenic effects have been demonstrated in animal experiments, the use of ibuprofen during pregnancy should, if possible, be avoided. In the limited studies so far available, ibuprofen appears in the breast milk in very low concentration and is unlikely to affect the breast-fed infant adversely.
- Safety and efficacy for use as an antipyretic in infants younger than 6 months of age, or for use as an antirheumatic in infants younger than 12 months of age, have not been established.
- Caution is recommended in geriatric patients, who may be more likely to develop adverse hepatic or renal effect with the medication and in whom gastrointestinal ulceration or bleeding is more likely to cause serious consequences, including fatalities.

Pregnancy and Lactation:

Pregnancy:
- The use of ibuprofen should, if possible, be avoided during the first 6 months of pregnancy.
- During the 3rd trimester, ibuprofen is contraindicated as there is

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there is a risk of premature closure of the foetal ductus arteriosus with possible persistent pulmonary hypertension. The onset of labour maybe delayed and the duration increased with an increased bleeding tendency in both mother and child.

Lactation:

- ibuprofen appears in the breast milk in very low concentration and is unlikely to affect the breast-fed infant adversely.

Effects on Ability to Drive and Use Machine:

Following treatment with ibuprofen, the reaction time of patients may be affected. This should be taken into account where increased vigilance is required, e.g. when driving a car or operating machinery.

SIDE EFFECTS

Frequent incidences include gastrointestinal bleeding and peptic ulceration, other gastrointestinal effects - mild to moderate abdominal or stomach cramp, pain or discomfort, heartburn or indigestion, nausea, diarrhoea, constipation, decreased appetite or loss of appetite, skin rash.

Less frequent to rare incidences include headache, dizziness, nervousness, pruritus, tinnitus, depression, oedema, drowsiness, insomnia, blurred visions and other ocular reactions, hypersensitivity reactions, abnormalities of liver function test, impairment of renal function.

DRUG INTERACTIONS

Ibuprofen should be avoided in combination with:

Acetylsalicylic acid:

Unless low-dose aspirin (not above 75mg daily) has been advised by a doctor. Concomitant administration of ibuprofen and acetylsalicylic acid is not generally recommended because of the potential of increased adverse effects.

Other NSAIDS including cyclo-oxygenase-2 selective inhibitors:

Avoid concomitant use of two or more NSAIDs as this may increase the risk of adverse effects.

Ibuprofen should be used with caution in combination with:

Anticoagulants:

NSAIDS may enhance the effects of anti-coagulants, such as warfarin.

Antihypertensives and diuretics:

NSAIDs may diminish the effect of these drugs. Diuretics can increase the risk of nephrotoxicity of NSAIDs.

Corticosteroids:

Increased risk of gastrointestinal ulceration or bleeding.

Anti-platelet agents and selective serotonin reuptake inhibitors (SSRIs):

Increased risk of gastrointestinal bleeding.

Cardiac glycosides:

NSAIDs may exacerbate cardiac failure, reduce GFR and increase plasma glycoside levels.

Lithium:

There is evidence for potential increases in plasma levels of lithium. Methotrexate: There is a potential for an increase in plasma methotrexate.

Ciclosporin:

Increased risk of nephrotoxicity.

Mifepristone:

NSAIDs should not be used for 8-12 days after mifepristone administration as NSAIDs can reduce the effect of mifepristone.

Tacrolimus:

Possible increased risk of nephrotoxicity when NSAIDs are given with tacrolimus.

Zidovudine:

Increased risk of haematological toxicity when NSAIDs are given with zidovudine. There is evidence of an increased risk of haemarthroses and haematoma in HIV(+) haemophiliacs receiving concurrent treatment with zidovudine and ibuprofen.

Quinolone antibiotics:

Patients taking NSAIDs and quinolones may have an increased risk of developing convulsions.

SYMPTOMS OF OVERDOSE AND TREATMENT

In children ingestion of more than 400mg/kg may cause symptoms. In adults the dose response effect is less clear cut. The half-life in overdose is 1.5-3 hours.

Symptoms:

Most patients who have ingested clinically important amounts of NSAIDs will develop no more than nausea, vomiting, epigastric pain, or more rarely diarrhoea. Tinnitus, headache and gastrointestinal bleeding are also possible. In more serious poisoning, toxicity is seen in the central nervous system, manifesting as drowsiness, occasionally excitation and disorientation or coma. Occasionally patients develop convulsions. In serious poisoning metabolic acidosis may occur and the prothrombin time/INR may be prolonged, probably due to interference with the actions of circulating clotting factors. Acute renal failure and liver damage may occur. Exacerbation of asthma is possible in asthmatics.

Treatment:

Management should be symptomatic and supportive and include the maintenance of a clear airway and monitoring of cardiac and vital signs until stable. Consider oral administration of activated charcoal if the patient presents within 1 hour of ingestion of a potentially toxic amount. If frequent or prolonged, convulsions should be treated with intravenous diazepam or lorazepam. Give bronchodilators for asthma.

DOSAGE AND ADMINISTRATION

Take tablet(s) with water. Should be taken with meal or as soon as after taken a meal.

Adults and children over 12 years :
Oral, initial dose 1 tablet, then if necessary, 1 tablet every 4 hours.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30°C. Protect from light and moisture.

Presentation/Packing:
Film-coated tablet blisters of 10 x 10's

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